

Patience

The Patient filled a middle-spot between plain clinical hall and locked room; the fifteen-minute visit sort of area opposed to proper care. If it were up to them, the entire hospital would look brighter, better concealing patients' fears emboldened by pretense: *Your case is special and you must suffer this land. Your case is special and you must live.*

Life—pretending at life, better said—meant much more than doctors' ill businesses and ill wills (pun intended). Medicine was corporate to the core anyway. Not a charitable act. If it were up to the Patient, repeated for force, they'd change things. Oh, no, they'd do more than *change* things; they'd burn brutality to the ground and smile, bald and sore, as their broken legs waddled out worse than before, seizing the reversal of recovery so often preemptive of revolution.

But anyway, they couldn't exactly say these things aloud. Such proclamations would be treasonous. Right. Although their sad state permitted them expressly in all manners to murder, stomp, and torture, some modicum of decorum spilt over from childhood held them back; kept their crass tongue flicking but never hot outright. Cutting words, soft scratches—they dealt in these now, and revelled in their victims' sympathy-surprise, regarding them no doubt as the trapeze artist of the circle, complicit in such—such dangerous games before death; almost ogling them, swiping their eyes as transaction cards over the Patient's divoted collarbone, how beautifully it stuck out, telling of mined-dry hunger or even better, consumption.

The Patient dealt in spades. They allowed their great show to continue, and they enjoyed watching back. Interest in the terminally ill paid interest in appearing ill. Laid bare, this was double voyeurism and nothing more profound—but then again, no man of sense charges death with profundity and philosophy. No man forgets his object of prayer.

Upon the Patient's neck sat a blue lanyard with white print. *This* object severed their last nerve to genteel society, though they wore it anyway. Not to do otherwise. Loitering below the hospital trees (which bordered a real backwoods forest) was all else that kept them busy. Today they'd especially regretted skipping out, but their coming visit was important.

Once again, twice again, maybe thrice again as their slippers hit the floor, the Patient rehashed a list of ways to improve the lands on which they stood—no—*occupied* like an explorer, renouncing complaint as savagery.

First: those stretchers needed to move out of the way. They ruined the spaciousness crucial to human environments, and instead played a badly-acted soapbox show for no right reason. Also, also. Hallway stretchers were only useful when facilities took patients. Having them around empty gave the hospital a bad look.

Second: these complaints and the words that formed them began to birth rhythm in their hollow head, which meant the lights needed changing. Though the Patient only knew hospitals personally, not in any academic matter, they didn't see why one tonal shift would cause major problems. Accustomed eyes would soon re-acustom themselves. Complaints would meet the nearest trash can.

Twirling their thick, thick lanyard (hard on the knuckles like rope), the Patient sped down a hallway to the right and marvelled briefly at moles. Quite likely, this building was designed by them. Maintaining human intelligence was no easy feat.

Third: the concept of 'legal' and 'illegal' medicines meant squat. If even the cleanest substances were inexorably, inevitably diseased, causing cancer, among a myriad of others, then what was the point of distinguishing them? The Patient preferred quick deaths among the visions and flowers rather than... rather than whatever the hell legal death entailed, that being murder under bureaucracy; tragedies just tracing the thighs of ethics.

For another distracted moment, they saw themselves pasquinaded among pharmacy signs—tomatoes hurled at their sick, thin, weary neck—and they sighed, rounding another corner. Drugs were drugs and drugs were poison. Some poisons were worse than others, no doubt about that, but the general ambit at play was lawless. Undeserving of legislation. An age-old mystery, why doctors couldn't simply give them cocaine for the pain and kick them out into the snow.

Far from the middle-spot stood a bright purple door. Not royal purple, but gratingly close, festooned with stickers, old adhesive slathered over empty spots like acne scars. The Patient noted Mickey Mouse and Finding Nemo, all peeling, some faded entirely, and tried the door, staring with particular passion at Goofy above the handle. He'd always been their favourite.

The Patient worked the hinges, at which point a strange calm fell upon them. They barely noticed the change at first, for even this could not pinch the half-formed nerve endings in their hands. It would also jive that a piece of them had always *been* calm, kept on the back burner, and finally felt safe enough to find pause.

Few ways to describe the place's scenery other than, "special room for special children". The interior, complimenting the door, was bulldozed with bright orange paint and smelled like legal alcohol. Three stools were scattered about: one for the doctor, one for the parent, and one for the parent, rolled askew; urged by an invisible wind. Only the doctor's chair bore weight. On the furthest wall, a faulty tap drip-drip-dripped every other second, and on its adjacents a variety of important diagrams like *How to Brush Your Teeth* and *Strep Throat* (seriously?) hung on hooks.

Fourth: though this was more praise than criticism, patients would be much happier if they all returned to childhood. Even metaphorically. Somewhere in the Hippocratic Oath, it was

decreed that colourful walls did more for terminality than therapy—and doctors, when given the choice between treatment and *treatment*, should always choose the latter.

The Patient felt quite happy here. They also felt it proper to mention that the exact tile on which they stood was a perfect vantage point, and from there they could watch the entire room symmetrically like a god-emperor of illness. Like a child, even so, standing on a stool and proclaiming themselves the top of the world. Not *at* the top; just the top.

“You’re thinking wrong thoughts again, aren’t you?” said the occupied doctor’s stool.

“I’ve been thinking an awful lot,” replied the Patient, cocking their head to one side.

“That you have.”

The stool swivelled closer.

“Look at me for a moment.”

Poor, poor thing, the speaker (doctor, friend, occupant, object) sighed under their breath.

They wore typical fare; a white lab coat buttoned down to the bottom, silver-rimmed glasses with yellowing nose pads, and black sneakers, since slacks were expensive and white detracted from the absence of itself elsewhere. Nothing special about them, besides a slight despotism and slant in posture, was to be said. The Patient stared into their eyes. They went on until both shed tears and broke apart.

“What do you want this time?” asked the doctor, exasperated to untrained ears.

“Hospital looks lovely. As always.”

The Patient settled upon a waxy bed.

Fifth: there was an issue with humans who believed they had no issues. Those who assigned themselves perfection. The world itself was a problem, damn anyone preaching otherwise, and by extension each and every forsaken inhabitant occupying the problem-space had problems; id est, issues. Problematic content, id est propaganda, and problematic events, id est change, were some of the many problem-issues plaguing humans on a day-to-day basis, and with blinding overexposure to the hard bits, they grew shells themselves. Bland, white-bread examinations ruined whatever remained of the world’s intimacy.

Wiping their last tear, which had left a salt-track on their cheek, The Patient went on.

“I suppose you’d like to hear my symptoms?”

“The system can send them to me.”

“No, *no*,” they groaned, tugging at their lanyard, “I want you to write them down.”

Like snow, small shreds of wax paper fell onto the tiles. One would expect them to freeze the place up, but they did not, and died shortly after coming down. Nature hadn’t the strength to survive in such deprivation. Even doctors, animals themselves, willingly altered their natural states; a real sign of the times. They’d recently taken to acquiring psychotherapy certificates,

since medicine was nowhere near as lucrative as it used to be, and the pandemic of lost patients criticising hospital walls and uneven floors and any of the Patient's qualms magnified ten times over had become somewhat of a problem. Hospital psychosis, experts called it. The new pandemic. Hospital psychosis, as if one disease was not enough.

"Pen," promised the doctor, waving a blue stick that rattled, "and paper."—waving, in another hand, a spiral-bound notebook.

"Speak away."

The Patient nodded and began to half-disrobe.

For easy access in the medical sense, hospital gowns were open-backed, humiliating pieces of clothing. They fit oddly, loosely, and rubbed against the skin worse than wool, so untoward the body that it became meat. A floppy piece. A thing to be lamb-shorn with scalpels.

The Patient took control of themselves for a quick and precious instant, and in shedding all barriers to their condition, they enacted the old favourite performance. Maintaining eye contact, lanyard swinging around their neck like a loose sheaf of hair, they pointed to their chest, skin pulled taut.

"There's something inside of me, doctor," The Patient stressed.

"Ever since— ever since I can remember, I've been living a half-life. I've been walking around and marrying the love of my— I've been having children and working at the corner store like you recommended. All that. Because it would— because it would fix my issues with, you know, the *syndrome*. But nothing works. I'm back again."

Quite contrary to their criticisms earlier, The Patient found themselves empty of thought; water-filled, tipped ninety degrees toward the table. As they gestured here and there to their heart, or where they believed the heart existed, the doctor wrote notes about instability and rest cures and palliative care for impossibilities made flesh.

The Patient wanted to believe they had control over this. Over everything. With their seniority, an element of responsibility was at play— an urge to complain and dominate, dance and think those wrong thoughts their doctor so often discouraged.

Nervously, they stared down at a tuft of hair growing from their right nipple. This body was indeed theirs, but pretending otherwise never hurt. In a world where flesh cost so much and consciousness even more, they couldn't help separating the two for maximum profit. Put plainly, becoming non-identical to the brain and the parts that logically formed them. Finding the unsullied soul.

Guiding the doctor's hand above their stomach, right between two ribs, they said, "Maybe I just need a bowl of rice. Do you hear any arrhythmia? Any murmur?"

“No. No. Everything as expected. 70 beats a minute, perfectly fine.”

Per usual, the lub-dub was nowhere out of place. The Patient wondered whether, on the impossibly off chance that their heart hadn’t been spasming regularly, the doctor would have done anything. In lawless lands like these, no right aphorisms existed. No cure-alls.

Sixth: One would *think* that empathy was some sort of unshakable gift. Ten days earlier, the speakers blasted two code blues, one after another. The Patient almost, very nearly cried—but stopped short and went about their dailies. Sorely did they miss the days when endings were monumental things.

Disappointed, the Patient turned their mind inside out and back to the issue at hand. If treatment did not pass, they’d advocate for themselves.

“Do you do this to all your terminals?” They challenged, staring right into the whiteboard of the doctor’s face; half-surprised, half-relieved at what lack gripped them still. Each nod and note was sewn back, bleached.

“I don’t want to call Behaviour again.”

Seventh: Behaviour only came when the Patient lapsed, not relapsed. Sometimes the show curtains would get all moth-balled, and they’d have a bad night performing, though still pretty in fatalism, the budding of that word in clinicians’ mouths. Behaviour would sweep them up and gentle-parent them into learning their lines again. The sickly body, the rotting bedpan; here the Patient ripped themselves from filth and composed a smile. One eye, then the other. As rehearsed. They knew what they’d put out of their mind. By extension, they did not know it at all.

“Listen,” said the doctor, measured, “as always, I suggest curbing those anxious thoughts; they won’t do you or your blood pressure any good. Feelings of unfulfillment are... how do I say this kindly... *standard* in cases like yours.”

The Patient noted such language, and proceeded to fiddle with their lanyard some more.

“The best treatment, as mentioned earlier, is to stop Thinking.”

They blinked once.

“Stop
Thinking.”

These words were the force by which women died at the bedside and thread sewed their lips shut. These words were the impetus of medicine, churning upon fires upon fires of teleology. If murder serves then murder... does?

In spite of the pinched face that the Patient was quite obviously making, the doctor continued, “You should read those books on meditation...” until interrupted.

“No time,” cut the Patient.

In the seconds between their response and the doctor’s preparatory procedures, they deigned to try a new act for the occasion, diverting attention from what threads of plans were near-done; plans that would soon begin clawing, begging, at their skull soon enough. At some axis an ascendancy over illness, a self-directed inquiry, would come about— but no use dealing in hypotheticals. Action was now.

The Patient relished the draft coming over their pale back. The Doctor replaced old needle tips with new and straightened the paper on their examination table.

“Can you at least take your medication?” They half-begged.

The Patient thought about it, making a good show of the process.

“I can, but it tastes horrible.”

They stuck their tongue out.

“Six years ago, I went on a real nice hike by the coast,” they began.

“The water was so deep and blue, you could smell the colours—and the trees, oh, the *trees* were fairies dripping in fog. I drank salt like wine, I thought I had it made.

Seals bobbed out of the water every few minutes, and you know, I took photographs back then, so I went all the way out into the tide pool area with my Wellingtons and snapped them whenever their heads came up to say hello.

I got the tails and the heads, actually. I waited around the entire day until they slept.”

The doctor worked their notepad furtively, like a man doing his best to find the flowerbud, until they hissed at a hand cramp and peered, wide-eyed, at the crying Patient.

Comprehending the past behind cases such as these sincerely helped with treatment, what with the lack of licensed psychiatrists and the recent combining of medical disciplines. The physical and the mental, in their trained eyes, were little different. Seals, seals, *seals* could be a metaphor for gangrenous legs. Waiting was obviously representative of the Patient’s real-life wait; their death march.

“Point being, I’ve made a grave mistake. Do I want wings? Do I want to stop falling out of planes? Is there a cure for want? I’m really not certain there is.”

You believe that stories must mean something. Stories are stories.

The subtext of their words operated as above.

“There isn’t,” admitted the doctor, “but medication will manage your symptoms.”

For approximately ten minutes, the doctor engaged in regular practice, testing their reflexes and looking into their ear with a bright pointed light, taking their blood pressure and stealing their life with a fine needle. The Patient hadn’t much blood to give, though they said nothing about feeling faint and merely winced. Goodness was coming. Some light was soon to find this logistical mess of a place, as evidenced by their complaints. Soon.

“Everything looks normal thus far. As a matter of fact, you’re exceptionally healthy for a patient at your level of progression! Six more months at the least.”

Translated, the Patient would be prostituted in this hellhole for another half-year—maybe longer. A smile passed their lips, and they sank under great pressure and heat. What diamonds froze from their skin formed words.

“Ten years ago, I moved into my first apartment,” they began, crafting a space not unlike the examination table. Hard. Unyielding. Promising.

“I slept on a mattress on the floor. For dinner, I ate Chinese takeout and blew up the toilet, and I swear, swear to God, it felt like the first day of my life. Like nothing had existed before that series of moments, and if I were convinced otherwise, it was my mind tricking me. So I decided I’d start living properly from there on out, right? Right. Because I couldn’t just waste the immense, beautiful gift that had been given to me; the right to breathe and move and suffer.”

Hospitals were impersonal in every way that mattered, save for the delicate reminiscence of suffering; such a carefully attended-to task that one almost pitied the halls, their constant thought. The Patient drew two breaths and coughed,

“But I know I ruined it in the end. I know.”

The doctor’s eyes met the whorl of their hair. In two places it was parted. Slowly they approached, comfort needle in hand.

“Don’t touch me if you don’t need to,” snapped the Patient, back hard against the sticker-suffused wall. If they left, they’d leave a decal of their own. Rude memories—caterpillars mimicking snakes, snails with hollowed eye-stalks and worms for brains—were touch. Some hollow corner of their mind played a scene on rote.

“Hey. Hey. Can you check again? Inside?”

They did not want that scene screened before fresh eyes.

They *did*, however, work with their plan.

“I’m not performing surgery on you, if that’s what”—

“Yesterday, I decided I would do it,” they pressed, eyes flat as medical trays,

“Damn everyone, I would open myself up and figure out why I’ve been living a dream all these years, even through the good moments.”

There was some (?) truth to this. Hiking by the coast before relapse, they’d never seen seals. Waiting for their children to burst from sacs they’d felt perfect, abuzz with newness. Moving into their apartment, they’d cried and done little else. Today they dealt in opposites, and most cleverly had begun to trick themselves too.

If anything was certain, anything at all, sickness would not disappear with goodwill or prayer or counseling. Living through casts was their future, past, and Christmas, and hours spent roaming the halls, managing insults to keep the old wit sharp, would be their friend alone—unless they changed. Unless they dug to the root of the issue and pulled the bloody weed hard.

Before the doctor could muster some motivational quote, the Patient spoke loud and clear. “This hospital is a shithole, doc. This hospital... I opened myself up yesterday, doc. I closed my eyes and took a knife to my chest and slit the skin from here”, cuing an overblown gesture, “to *here*.”

The Patient had dropped their gown to the floor. Now fully bare, they referred to nothing at all; genitals eviscerated, a mound in place. Two small dots, irritation from monitor pads, were the only notable differences from the last examination, and the last, and the last before that. Sick folk did well with shame, being burlesque dancers, performing special services before panels of lechers. Nakedness was nothing.

“There isn’t a scar,” said the doctor.

“No. It healed just fine because I closed it up.”

Humouring them, the doctor replied, “Take your medication.”

For quite some time, both parties frowned at one another. Tension, for which The Patient took sole responsibility, hung low. When one died or found the answer, whatever came first, the pair could always become good friends and escape, breaking the genie bonds of fund that tied medical professionals to hospitals and—

“Fly off to Florida, if you so wish. Two pills a day with food?”

“Two pills a day. With food. What about Florida, now?”

“...Easy.”

The noise stopped in lieu of an easy silence punctuated by cursory glances, quick cligne-d’oeils and frog-blinks.

With no distractors to sever them from the world, The Patient turned their mind to disgusting things again. Hidden hospital noises, like the radiator’s hum; an urgent call, no doubt from the surgery two floors down; a gentle trickling of blood from another surgery, where a

fourteen year-old boy was under for his entry-exit gunshot wounds; nurses and their coffee cups of dubious origin; sections of the hospital completely blocked off, covered in vomit.

Breaking rules would be unwise but fun. So very fun.
In this manner of judgement did their resolve finally steel.

“I’ll get out of your hair and take my medication *if* you’ll do me a favour,” The Patient suggested, leaning in close.
“A favour...?”

Distractions. Not only were they naked and sickly, skin so thin the veins coloured them blue, but their words flowed like art, and what remained of their half-true stories laid careful brushstrokes for detail to come; demands like, “Give me a kiss,” or, “I want a therapy dog.” The Patient’s eyes darted to an unmanned scalpel. How weak it looked on the desk alone.

“Lend me that for one”—

Eighth: doctors never listened to their patients. They assumed, or were taught in medical school, that reported pain bore not on physicality. To them, women were superhumans and peasant men deserved crowns. Sickly, terminal in-betweeners (like theirs truly) deserved sufferance to balance the scale. Tales being fact, fact being tales, doctors dealt in topsy-turvy philosophies and would, like economists, bring about order whenever possible.

One, one...? The Patient’s complaint had gone on too long. One *what*, now?
Lunging ahead, they grabbed securely onto the dull end of the scalpel and tore their hand back, whip-quick.

—“Moment.”

Even better than burning this place down was advocating for oneself. If matters rested solely in the doctor’s hands, then The Patient would die in ages without recourse, believing life to be the only thing worth living for.

As much as power play attracted them, they’d grown restless. Meditation didn’t help and neither did medication. Look, those eyes absent of pupils, slate of feeling. Touch, that starched white coat. Insults— at least five every walk. Stretchers, lights, legality, decorations, wrong-right, the Behaviour nurses.

“I’m going to call Behaviour,” threatened the doctor, who rose to reach for a dial-up phone.
“You’re not calling Behaviour.”
Beep. Beep. Beep.

“You’re **not**.” The Patient cut into that awkward space between sitting and standing where bodies are weakest and most prone to surprise. Masking the sharp end of their scalpel, sane in

some ways still, they mustered all their strength and sent the doctor toppling over, legs crumpled, head dashing on a table corner along the way.

In five seconds, change had come.
Change was a matter of purpose.

Like they'd mentioned (the one full truth among many halves), some vital organ had quit their chest or never been made. What sounded was a liar, and what the doctor loved was a show.

Nobody would believe them, though, unless they exhibited clear, scientific proof of their lack, hence the need for force.

Force, beeped the fearful monitors a-clatter as The Patient pushed their doctor to the ground— as the white-coat's head, suddenly very human and very delicate, cracked upon a metal edge.

Without pause, they leaned down to measure each harrowed, slow-slow-slowed breath and confirm the final sink. They moved away from their work gasping, fighting for words. Stepping over the doctor, they reached high for the cabinet above the room's sink. It was unlocked.

Blood wet the bottoms of their slippers. Head wounds *did* tend to bleed.

On the backside of the cabinet door was a mirror, and on the inside was a profusion of lethal pills labelled according to science: skulls for danger and big red circles for impact. Today, The Patient did not feel like dying. They had just killed the only person they knew. It would be wrong to melt into the mess.

Carefully, they brandished their scalpel and brought the point to their bare chest. How strong and loud their magic pumped under the skin. They almost thought it a betrayal to let anything out— but they *had* to. This was the point.

This was the purpose.

The Patient screwed their eyes shut, for the job would not be done any other way, and dragged the weapon's point down the length of their chest. A horrid tearing noise screamed itself hoarse— sounded so strangely intimate that the act was perversion. The Patient had come here to make peace before death, funnily enough. Hellish shrieks *really* didn't help the goal along.

Floaters, bathing in the screwed-shut night of their eyelids, urged them to reconsider whether this was the right way to go about finding peace, synthesising peace. Perhaps the real thing existed in an upper-floor laboratory and not here, and their suggestion about childhood

pleasures was objectively true for everyone except them— and oh, they'd saved the entire world. All the others.

Ninth: Hospitals made things irrationally complicated.

The ripping continued until a suitable gash had opened, just as long as what they'd described to the doctor. Though their eyeballs were tight and near-glued to their coverings, The Patient understood that blood should have begun falling, as it should have been the first time when they'd felt too scared to look. There was a mere coldness, wind open to organs (or the lack thereof). Viscera chilled and slowed, they collected themselves and began breathing steadily again.

Like that poor, sad king named Charles the Second, no red did run in their veins. Like him, too, they were born of wrongful union and suffered long for it. A self-pitying phrase, "and yet I was still..." ghosted The Patient's mind. Past aside, present cast away, future set, they hadn't much to look forward to besides the reveal; the final proof of their vital lack, which would come if they only opened their eyes.

And yet I was still—

Still what, then? Scared? After examinations far more invasive than this, far more robbing of agency, they hadn't anything to fear.

Regrets abounded, but so did they for everyone.

Even normal, healthy bodies.

Restless, the floaters behind their eyelids began to writhe. The Patient knew they'd eventually have to look— if they wanted to understand themselves, that was. A necessary act of bravery; a medical marvel in the making. *Look, this form with no soul, no heart, no duodenum*, the board would have said, rushing over the poor doctor's body, clamouring to get a good look. Performing without them around was difficult.

The Patient tried and failed several times again. Their muscles wouldn't cooperate.

Cold, irregular heat pulsed from the wound, stumbling drunk. Whatever sat inside was no ordinary organ. They swore it on their life. Human hearts were warm and constant; none of this gear-grinding mess. Bearing the weight was not painful so much as frustrating; hard to ignore, just as pulled muscles nagged at the mind— being actually hurt in the smallest, dumbest, most useless way but *feeling* hurt like nothing ever.

After centuries, The Patient managed to pry one eye open. The left. Looking down was a task still too great for them, and their neck had been barred with lock and key. They could only stare straight at their own face in the mirror, all else blurred, with its sharp cheeks and sunken-dish eyes.

Just below, nothing. A lack of form at which they could not stare for long. Stumbling away from the mirror, they hopped past the doctor's body, which laid in the way of them and a wide-open door. Behaviour hadn't come. Doubtful they would ever.

Tripping over the mess of blood that had grown half-sticky, half-slippery on their soles, The Patient filled a slightly darker middle-spot. Hospital hallways looked the same. Where they'd started was where they currently stood. Stretchers, mobile x-ray machines, and mobility bars dotted the horizon; a real pretty picture. Elaboration would have made a good tenth, but— not then. Not then.

The Patient grasped their lanyard, fighting hard for presence.
At the same impossible speed tripped their not-heart.

Upon further investigation, base control could be exercised over its contractions, speeding, slowing and bending to coherent rhythm. The Patient almost thought to make music until the whole thing proved too draining. Despite (or due to) experimentation they finally sank, resigned to fate, knees hard on the floor.

They groped around for another complaint, but found none in the cave of their chest. It was evident that the hospital would not burst aflame, and they would go on living life with the inexorable pain that so often dragged them down. The journey would not be learned from, their talks had wasted time— engendering a worthlessness they hoped to impress upon others— and despite some possibly valuable insights,

In fact nothing had been done
At all.